

# Dermosol<sup>®</sup> Cream

## Description:

An off-white to creamy color cream.

## Ingredient(s):

Each gram contains:

Clobetasol Propionate ..... 0.5mg

## Pharmacodynamics

Clobetasol Propionate shares anti-inflammatory, anti-allergic and antipruritic actions. Topical corticosteroids such as Clobetasol Propionate are effective in the treatment of corticosteroid responsive dermatoses primarily because of their anti-inflammatory, antipruritic and vasoconstrictive actions. However while the physiologic, pharmacologic and clinical effects of the corticosteroids are well known, their exact mechanism of actions in each disease are uncertain.

## Pharmacokinetics

The extent of percutaneous absorption of topical corticosteroids including Clobetasol Propionate is determined by many factors, including the vehicle, the integrity of the epidermal barrier and the use of occlusive dressings.

Clobetasol can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin may increase percutaneous absorption.

Once absorbed through the skin, topical corticosteroids enter pharmacokinetic pathway similar to systemically administered corticosteroids are bound to plasma proteins. Corticosteroids are metabolised primarily in the liver and are then excreted by the kidney. Some of the Clobetasol Propionate and its metabolites are also excreted into the bile.

## Indication(s):

Clobetasol Propionate is indicated for short term treatment of inflammatory and pruritic manifestations of moderate to severe corticosteroid responsive dermatoses, eg. psoriasis (excluding wide spread plaque psoriasis), recalcitrant eczemas, lichen planus, discoid lupus erythematosus and other conditions which do not respond satisfactorily to less active steroids.

## Dosage and Administration:

Should be applied with gentle rubbing to the affected skin areas twice daily, once in the morning and once at night. Therapy should be discontinued when control is achieved. In the more responsive conditions, this may be within a few days. Rarely, gradual withdrawal of therapy or supplemental systemic adrenocorticoid therapy may be required to avoid symptoms of steroid withdrawal.

Clobetasol Propionate is potent and therefore treatment must be limited to 14 days and amounts greater than 50g per week should not be used. If no improvement is seen, reassessment of the diagnosis or referral may be necessary. This cream is not to be used with occlusive dressings.

## Precaution(s):

1. Long-term continuous therapy with Clobetasol Propionate should be avoided, particularly in infants and children, in whom adrenal suppression occurs readily.
2. If Clobetasol Propionate is required for use in children, treatment should be reviewed weekly and it should be noted that the infant's napkin may act as an occlusive dressing.
3. The face, more than other areas of the body, may exhibit atrophic changes after prolonged treatment with potent corticosteroids. This must be borne in mind when treating such conditions as psoriasis, discoid lupus erythematosus and severe eczema. If possible, courses should be limited to 5 days. If applied to the eyelids, care is needed to ensure that the preparation does not enter the eye, as glaucoma might result.
4. Topical corticosteroids may be hazardous in psoriasis for a number of reasons including rebound relapses, development of tolerance, risk of generalised pustular psoriasis and development of local or systemic toxicity due to impaired barrier function of the skin. If used in psoriasis, careful patient supervision is important.
5. Appropriate anti-microbial therapy should be used whenever treating inflammatory lesions which have become infected. Any spread of infection requires withdrawal of topical corticosteroids therapy and institution of suitable systemic chemotherapy. Skin should be cleaned before fresh dressing is applied.

6. Topical preparations of Clobetasol should not be used in pregnant patients or in patients who are planning to become pregnant. Topical preparations of Clobetasol should not be used during the course of breast feeding.

**Contraindication(s):**

Clobetasol Propionate is contraindicated in patients who are hypersensitive to Clobetasol Propionate, to other corticosteroids or to any ingredient in their preparations.

Contraindicated in treatment of rosacea, acne and perioral dermatitis, routine gingivitis, perianal and genital pruritus. Dermatoses in children under one year of age, including dermatitis and napkin rashes. Also skin lesions caused by infection with viruses (eg. herpes simplex, chickenpox), fungi (eg. candidiasis, tinea) or bacteria (eg. impetigo).

**Side Effect(s) / Adverse Reaction(s):**

The most frequent side effects include burning sensation, irritation and itching. Less frequent side effects are stinging, cracking, erythema, folliculitis, numbness of fingers, skin atrophy, telangiectasia, pigmentation changes, hypertrichosis and hypersensitivity.

Extended use of large amounts or treatment of large areas may cause sufficient systemic absorption to produce symptoms typical of hyperactivity of the adrenal cortex, with moon face, sometimes with hirsutism, buffalo hump, flushing, sometimes leading to a fully developed Cushing's syndrome. Provided the weekly dosage is <50g in adults, any pituitary-adrenal suppression is likely to be transient with a rapid return to normal values once the short course of steroid therapy has ceased. The same applies to children given proportionate dosage.

In rare instances, treatment of psoriasis with corticosteroids (or its withdrawal) is thought to have provoked the pustular form of the disease (see precautions).

**Symptoms and Treatment for Overdosage, and Antidote(s):**

If irritation develops, Clobetasol Propionate should be discontinued and appropriate therapy instituted. Chronic topical overdosage may result in the features of hypercorticism and treatment is symptomatic, supportive and consists of discontinuance of topical corticosteroid therapy. Gradual withdrawal of the preparation may be necessary. For acute oral overdose, treatment consists of dilution with fluids.

**Shelf-Life:**

3 years from the date of manufacture.

**Storage Condition(s):**

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

**Packing(s):**

Plastic jar of 100g and 500g (for export only).

Aluminium tubes of 10g and 15g.

Aluminium tubes of 20g(for export only).



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